

Supplementary public-feasibility outputs

If the separately registered public CXL-only experiment is executed, supply its own screening/missingness table, five-predictor model performance table, calibration plot and coefficient table. Use the label public CXL-only, nominal one-year CDVA throughout. Report source patient-ID counts, adult/quality exclusions, CXL protocol mix and unresolved date/laterality limitations. These results do not populate the KKESH treatment-comparison tables or establish external validation of its model.

3 Output verification and references

Reporting basis: Collins GS et al. TRIPOD+AI. BMJ. 2024;385:e078378. <https://doi.org/10.1136/bmj-2023-078378>

Development/evaluation appraisal: Moons KGM et al. PROBAST+AI. BMJ. 2025;388:e082505. <https://doi.org/10.1136/bmj-2024-082505>

Analysis definitions and denominator rules are governed by the accompanying Statistical Analysis Plan, version 1.0.

- Reconcile screening and analysis counts to the saved population and unique coded patients.
- Check every unit, visual-acuity conversion and paired-change sign against the transformation specification.
- Recalculate representative summary cells and model metrics independently from saved analytical data/predictions.
- Confirm no patient appears in both training and test partitions, including bootstrap duplicates.
- Show missingness, dates, failed resamples and analysis limitations; all unavailable results say not run or not estimable.
- Inspect rendered tables and figures for clipped text, unreadable labels, fabricated precision and accidental identifiers.